



Vaccine Injury Profile

10th September 2026

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About

COVERSE is Australia's national charity dedicated to supporting Australians who have suffered harms from COVID-19 vaccines. It was founded in 2022, and its mission and activities can be found on its website at coverse.org.au.

One of the most important aspects of **COVERSE**'s work has been numerous submissions to public inquiries in Australia, especially federal Australian Government and Parliament of Australia inquiries relevant to the interests of **COVERSE**'s constituency. These submissions can all be found at coverse.org.au/submissions.

In order to generate data to assist with understanding the issues and needs of the COVID-19 vaccine-injured and bereaved community in Australia, the organisation established an online Vaccine Injury Profile. This Profile allowed users registered on the **COVERSE** website to provide details of their personal experiences as people who had suffered serious adverse events following COVID-19 vaccination. This captured information about vaccine doses, symptoms, medical diagnoses, treatments, financial and social issues, and a range of other aspects. All of this helped inform **COVERSE**'s national advocacy efforts and the preparation of its public submissions and other content.

This document contains summary statistics and analysis of **COVERSE**'s Vaccine Injury Profile data.

It is not a scientific paper. It has not been peer-reviewed, nor has the Vaccine Injury Profile been granted ethics approval for use in research. Responses are not available for public inspection, as they were collected under strict privacy conditions that disallowed this kind of disclosure of individuals' information.

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- Only non-commercial uses of the work are permitted.
- No derivatives or adaptations of the work are permitted.



Methodology

The Vaccine Injury Profile asked more than 40 questions related to Australian individuals' experiences with COVID-19 vaccine injury, including health as well as social and financial aspects. The entire list of questions can be [viewed online](#).

The list of questions was developed based on several considerations, including the experiences of international counterparts in conducting their own surveys (including [Germany](#), the [UK](#), and the [USA](#)), information gleaned from social media support groups, and **COVERSE**'s need for Australian data to guide its advocacy efforts.

In order to make the Profile as easy as possible to complete, **COVERSE** designed the Profile with many of the questions (especially the qualitative ones) not being required to be completed. Not all individuals who started the Profile completed all required questions, and not all completed the optional questions.

Note that the underlying denominator, N, we used for statistical analysis varies from question to question, depending on whether the question was compulsory, how many people answered the question, and demographic specificities (e.g. biological sex). This allowed us to utilise larger or more appropriate data in the analysis of each question. Throughout the analysis the relevant N used for each calculation will be provided. Where we utilised all Profile responses we depict the denominator as N_{all}. Where we incorporate only those responses from individuals who answered a particular question, we depict, for example, N₁₃₈ to indicate we only used 138 responses. We appreciate that this is not a usual methodology for statistical analysis of this type, however our aim was to extract as much useful information as we could from the data we have, within reason, to guide our advocacy.

Most summary data is presented as a percentage of the given N, and where appropriate we also provide calculated values (using standard Microsoft Excel functions) such as median, average, standard deviation (σ), confidence interval (CI), etc.

Data was collected between 23rd September 2022 and 21st September 2025.

AI disclosure statement

This report was authored by **COVERSE** personnel and was not drafted or written by generative artificial intelligence (AI) tools.

Such tools were, however, used to assist with analysis of data and responses. In particular, [Microsoft 365 Copilot](#) was utilised to augment analysis of open-ended text responses. Those responses were manually cleaned (fixed typographic errors, ensured consistent disease naming, removed any and all personally identifiable content, etc.) before being entered into Copilot, and we ensured that Microsoft policies did not allow this data to be stored or used for any purpose (including the training of models) other than our inputs and requested analyses. Use of Copilot is explicitly disclosed, where relevant, in the Results section.

Results

Demographics

A total of 594 individuals began answering the Vaccine Injury Profile (N_{all}), with 525 having completed all required questions. Within N_{all} responses, 65.0% indicated being female, and 35.0% being male.

Age values were calculated by subtracting the year of birth from the date the respondent began completing the Profile. The average age of N_{535} respondents was 50.4 ± 14.0 (95% CI 49.2-51.6). The average age of females was 51.4 ± 13.4 (95% CI 50.3-52.5), and average age of males was 48.3 ± 15.0 (95% CI 47.0-49.6). The age distribution is presented in Figure 1.

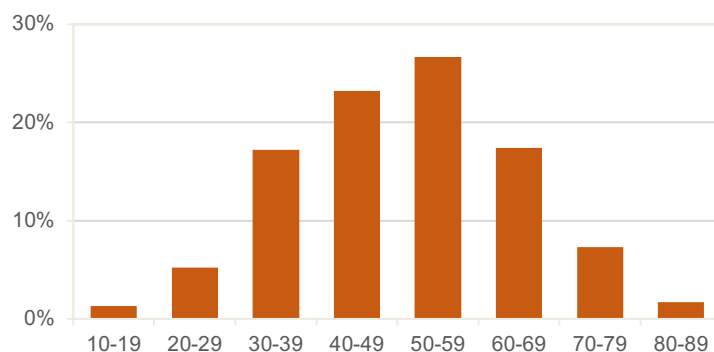


Figure 1. Age distribution of respondents.

All of Australia's States and Territories were represented among the respondents, with the proportion from each region displayed in Figure 2.

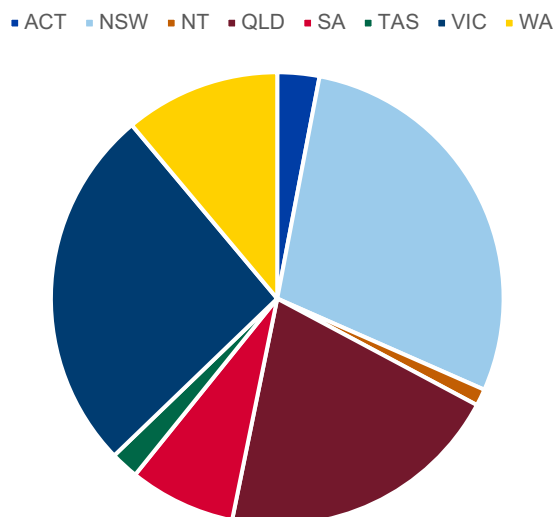


Figure 2. Distribution of respondents across Australia's States and Territories.

When asked if they would be interested in participating in research involving COVID-19 vaccine-injured people, 83.7% indicated that they would.

COVID-19 vaccination history

Respondents were asked about the COVID-19 vaccine doses they received, including whether they experienced an adverse reaction to each. A total of 1,530 doses were recorded in the Profile, of which 62.3% were followed by an adverse reaction. Results for each dose are summarised in Figure 3.

dose <i>n</i>	N (received dose <i>n</i>)	% (reacted to dose <i>n</i>)	median date of receiving dose <i>n</i>
1	594	62.0%	23 rd August 2021
2	505	71.5%	19 th September 2021
3	273	65.2%	19 th January 2022
4	117	35.0%	15 th July 2022
5+	41	12.0%	26 th April 2023

Figure 3. Information about COVID-19 vaccine doses received and reacted to.

Figure 4 depicts the date distribution of doses reported by respondents, and Figure 5 provides a percentage breakdown of the brands of COVID-19 vaccines that respondents received. The percentages of both figures are out of N_{all} respondents.

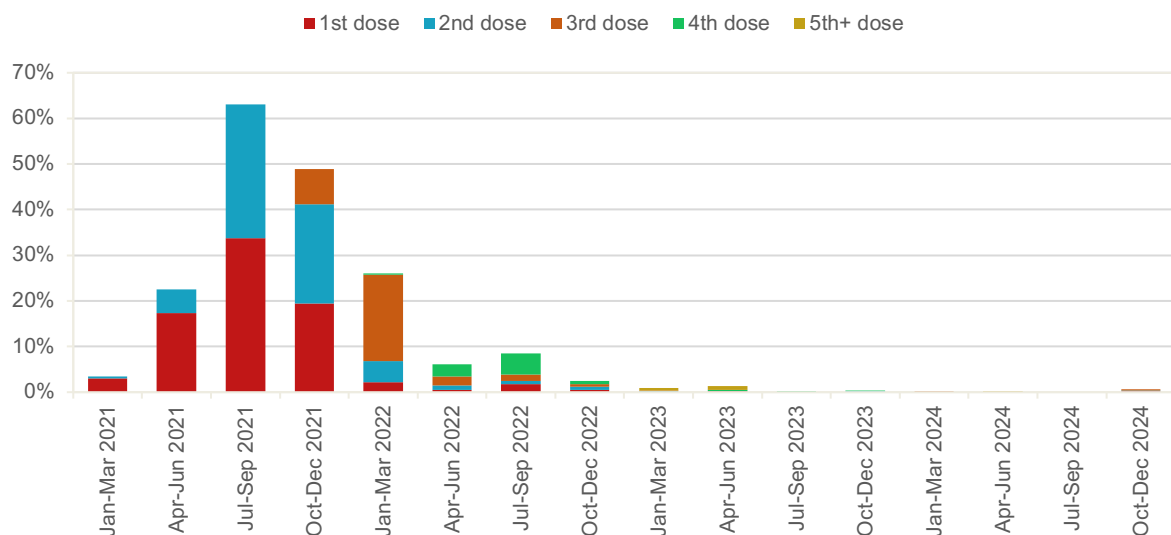


Figure 4. Date distribution (by quarters) of all doses reported by respondents. Note that not all respondents were able to indicate exact dates for each of their vaccination doses.

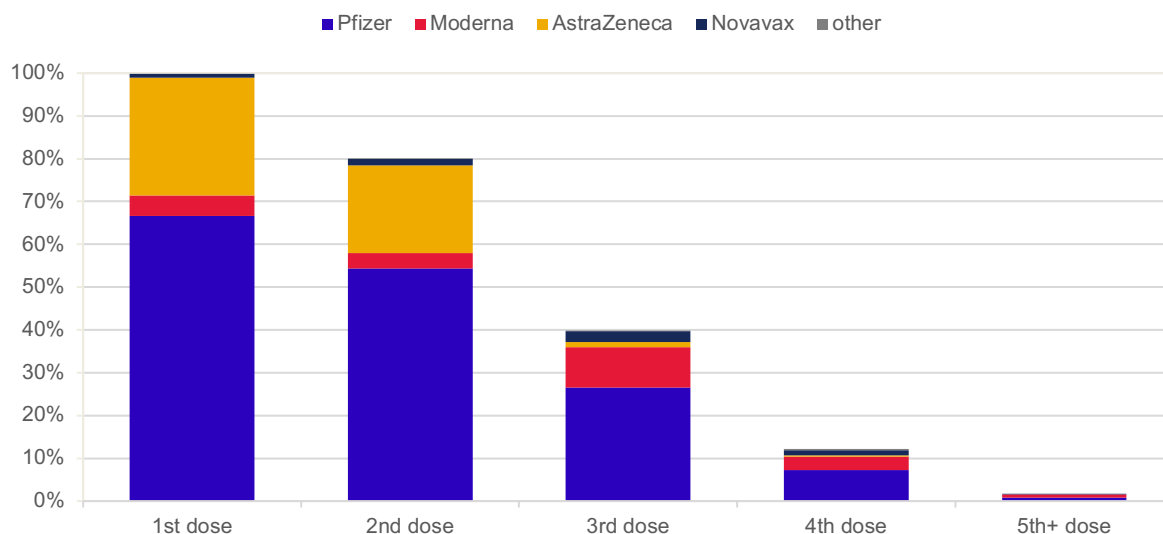


Figure 5. Relative distribution of COVID-19 vaccine brands among respondents.

Time to symptom onset varied, however the most commonly reported periods were within 1-12 hours and 1-7 days. Figure 6 shows the percentage of N_{all} respondents who reacted within each depicted time period.

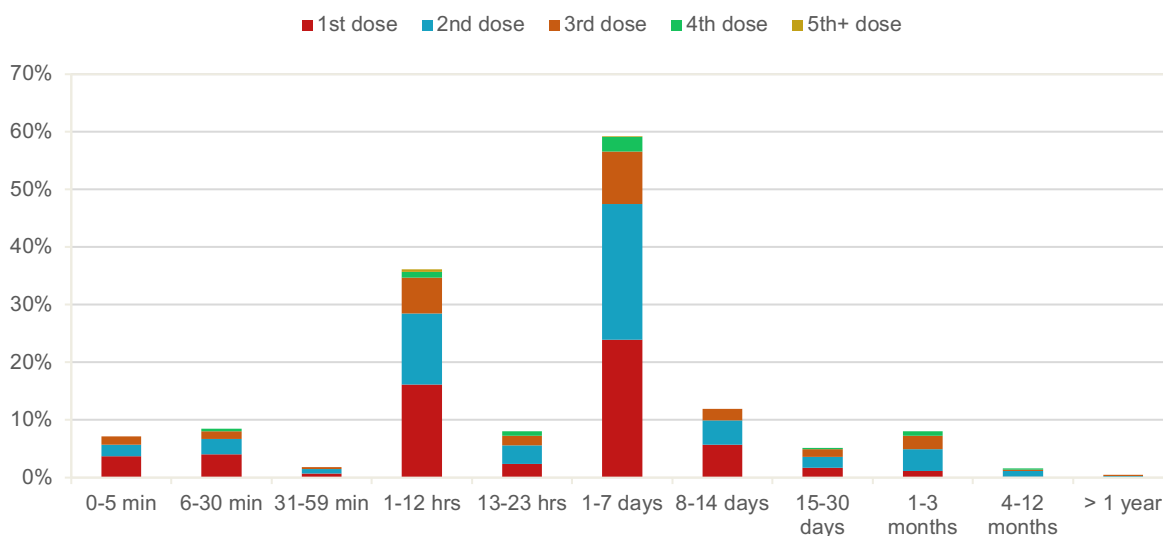


Figure 6. Time to symptom onset after vaccination.

Of N_{318} respondents who specifically indicated that they got subsequent doses after reacting to an earlier dose, 78.3% indicated that their health worsened, 2.5% felt better, and 19.2% had no change in their symptoms.

N_{263} indicated whether or not they were subject to a workplace COVID-19 vaccine mandate for the dose that they became injured by, of which 51.0% reported that they were formally mandated to receive that dose while 8.4% reported that while they were not formally mandated they were bullied or coerced by their workplace to receive the dose. Note that this particular question was added to the Profile some time after the Profile was initially opened for submissions, hence why we could not capture a greater proportion of all respondents.

Adverse reaction / symptom details

For a collection of 71 symptoms, we asked respondents to indicate if they had experienced each as part of their post-vaccination symptoms, and how severe these symptoms were *at their worst* (minimal, mild, moderate, moderately severe, or severe). Figure 7 presents an overview of these results, from the N₅₇₇ people who completed this question. Note that N/A indicates that the respondent specifically indicated that the particular symptom was not applicable to them, however it is assumed that no choice of N/A or symptom severity also indicates that the symptom was not applicable.

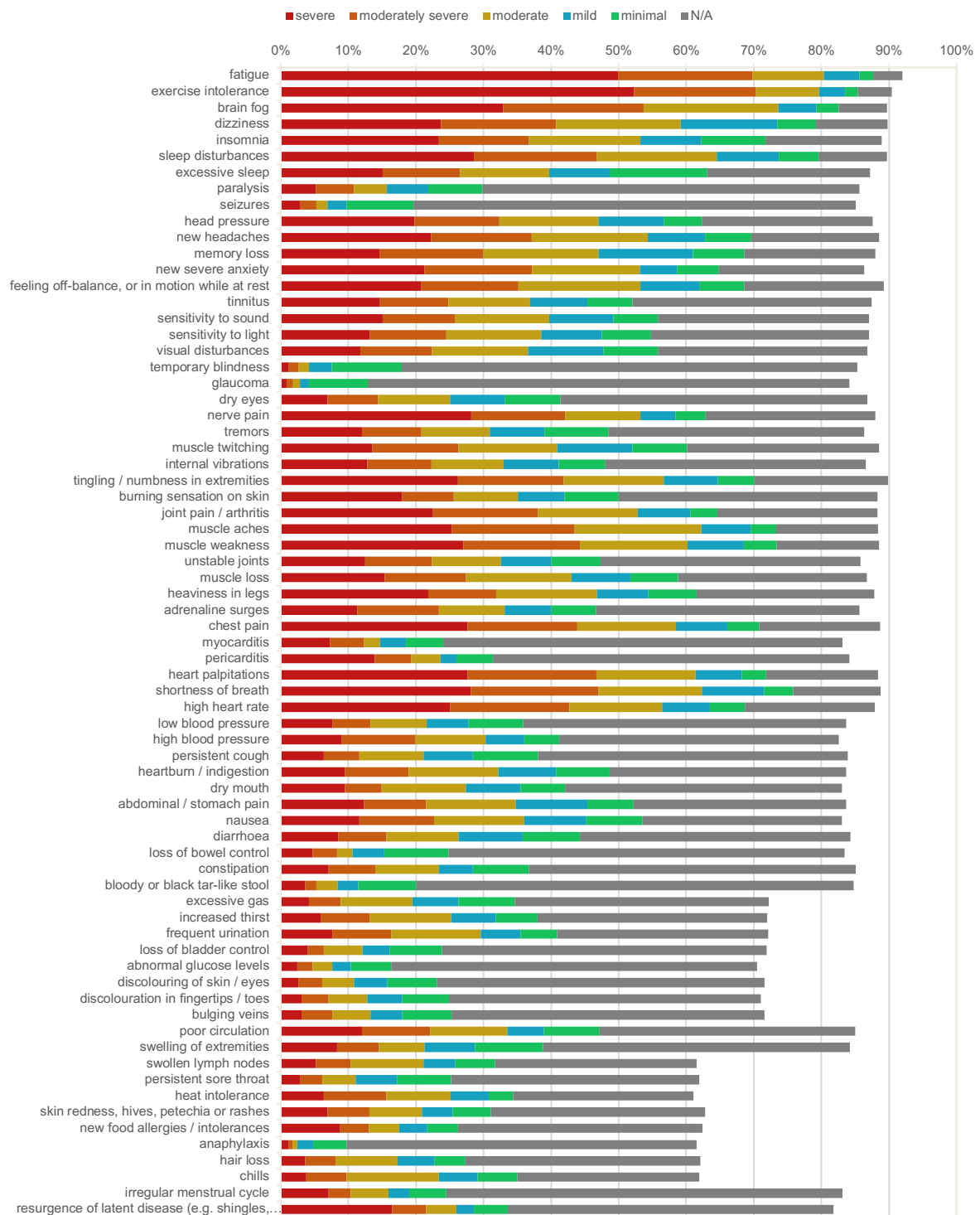


Figure 7. Post vaccination symptom severity at their worst.

In addition to these pre-defined symptoms, we also asked about any other symptoms that people experienced. There was a wide range of free-text responses from N₃₀₀ people, which included diagnoses, test results and functional capacity as well as symptom descriptions. We combined these other responses with the pre-defined symptom responses and summarised the combined N₅₆₅ records with assistance from Copilot:

Most respondents reported symptoms spanning multiple organ systems simultaneously. Symptom burden was frequently high, with many respondents describing moderate to severe impairment. The responses describe a highly heterogeneous but recognisable pattern of multi-organ symptomatology, dominated by fatigue, neurological dysfunction, cardiovascular involvement, musculoskeletal pain, immune/allergic manifestations, and autonomic instability.

The top 10 most commonly reported symptoms, that appear across the majority of responses, encapsulate:

1. Fatigue
2. Exercise intolerance / post-exertional malaise
3. Brain fog / cognitive impairment
4. Dizziness / balance disturbance
5. Sleep disturbance
6. Headache or head pressure
7. Cardiac palpitations / abnormal heart rate
8. Chest pain
9. Muscle and joint pain
10. Peripheral nerve symptoms

Recurring multi-symptom constellations that stand out across many of the responses are:

- Multi-system neuro-autonomic symptoms: brain fog, dizziness, balance problems, palpitations, heart rate variability, heat intolerance, chills, adrenaline surges, bladder/bowel control disturbances. Symptoms span central, peripheral, and autonomic nervous systems, and are often described as fluctuating but persistent and disabling.
- Severe fatigue with post-exertional malaise: profound fatigue, exercise intolerance, cognitive worsening after activity, need for prolonged rest or sleep. Functional limitation is central, rather than organ-specific, and the symptom burden is often reported as life-altering with reduced capacity for daily living.
- Inflammatory cardio-respiratory symptoms: chest pain, myocarditis/pericarditis, shortness of breath, persistent cough or air hunger.
- Neuropathic pain and sensory dysregulation: burning skin, tingling, numbness, tremors, internal vibrations, facial neuralgia, limb weakness, sensory loss. Descriptions are classic for peripheral and small-fibre neuropathic phenomena, and are often severe and constant.
- Immune / inflammatory reactivation and hypersensitivity: new food allergies, anaphylaxis, rashes, petechiae, swelling, swollen lymph nodes, reactivation or emergence of autoimmune or inflammatory diagnoses. Combines dermatologic, lymphatic, allergic, and systemic inflammatory features.

The dominant theme is not any single organ-related symptom, but a systemic illness characterised by profound fatigue with exercise intolerance and widespread neuro-autonomic dysfunction, around which cardiovascular, musculoskeletal, immune, and gastrointestinal symptoms cluster. The symptom patterns most strongly resemble established multi-system neuro-immune illnesses such as ME/CFS, dysautonomia, and small-fibre neuropathy, often with overlapping inflammatory or hypersensitivity features, rather than any single well-bounded disease entity.

Conversely, Copilot pointed out that patterns *do not* resemble: isolated psychiatric illness, single-organ diseases, acute self-limited inflammatory reactions, or traditional degenerative neurological disorders alone.

While data are all self-reported, Copilot also notes that the dataset shows high internal coherence, cross-participant convergence, symptom-cluster stability, and severity-graded continuity, which together indicate a strong and robust syndrome-level signal.

Figure 8 provides the breakdown of which side of the body people felt their symptoms were more dominant.

side	% respondents (N _{all})
left	21.5%
right	9.3%
varies	16.8%
no difference	29.6%
N/A	22.7%

Figure 8. Dominant side of symptoms.

Of N₅₆₄ respondents, only 7.6% indicated that their health was definitely improving, with a further 21.3% indicating that they were slowly improving. Contrasting these numbers, 26.8% say they were not improving, 17.6% were actually getting worse, and 26.8% were experiencing ongoing fluctuation in their health.

Of those who were improving, slowly improving, or fluctuating, N₁₉₂ indicated that the median time before they began to experience symptom improvement was 8 months. The distribution of responses is shown in Figure 9.

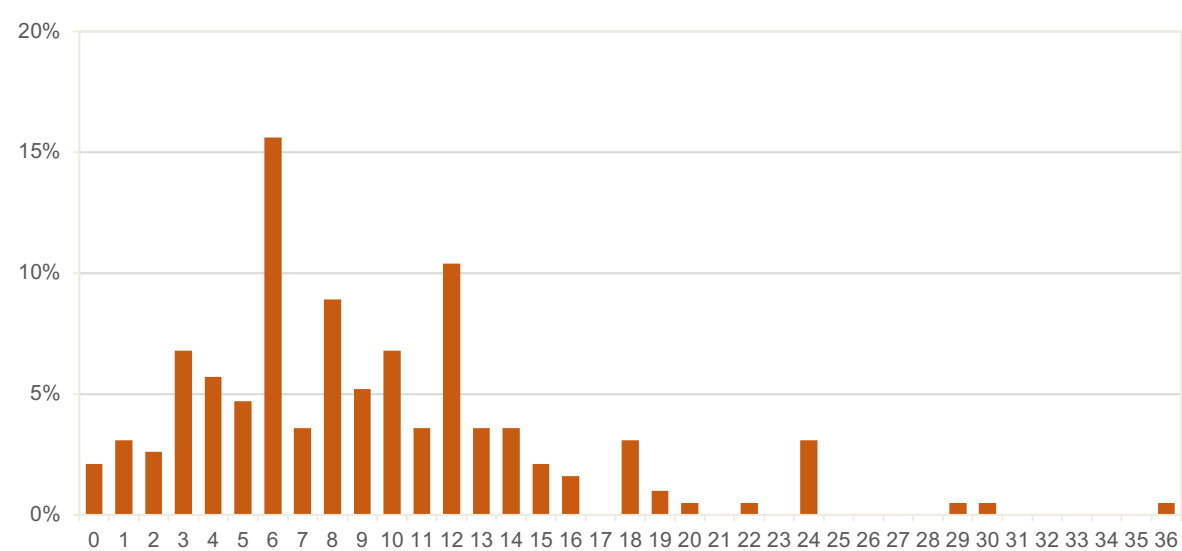


Figure 9. Months until improvement in symptoms.

Healthcare access, diagnostics and treatment

58.4% of N₅₄₆ respondents indicated that their usual GP recognised that they had experienced an adverse event following their COVID-19 vaccinations.

Of N₅₄₆ respondents, 51.5% were able to confirm that their adverse reaction had been reported to Australia's Therapeutic Goods Administration (TGA, the national drug regulator responsible for vaccine approvals and adverse event monitoring). Further breakdown of this reporting is shown in Figure 10.

reported	% respondents
yes (self-reported)	37.5%
yes (reported by GP / specialist)	10.4%
yes (reported by hospital)	3.3%
no	31.3%
unsure	17.2%

Figure 10. Summary of AEFI reporting to the TGA.

Out of those who confirmed that their adverse event following immunisation (AEFI) had been reported, 12.4% of N₂₇₅ indicated that the TGA had been in contact with them about their reaction. Note, however, that the wording of this question in its original form was ambiguous, and was only later revised to ask specifically if the TGA had made contact to seek further information or arrange additional medical investigations. We suspect that the actual number of instances where the TGA ever made contact with reporters for further information is closer to 0%, based on our many interactions with members of our community and further descriptive comments about this topic in respondent narrative text responses. However, we are unable to separate responses to the earlier wording of our question from the later more specific version to verify this.

83.9% of N₅₄₆ respondents had been referred to a medical specialist for further investigation (responses also included allied health specialists such as psychologist, dietitians, respiratory therapists, etc.). Of the N₄₅₅ who indicated which specialists they had seen, the median number of distinct specialists seen was 2. The distribution of specialities is summarised in Figure 11.

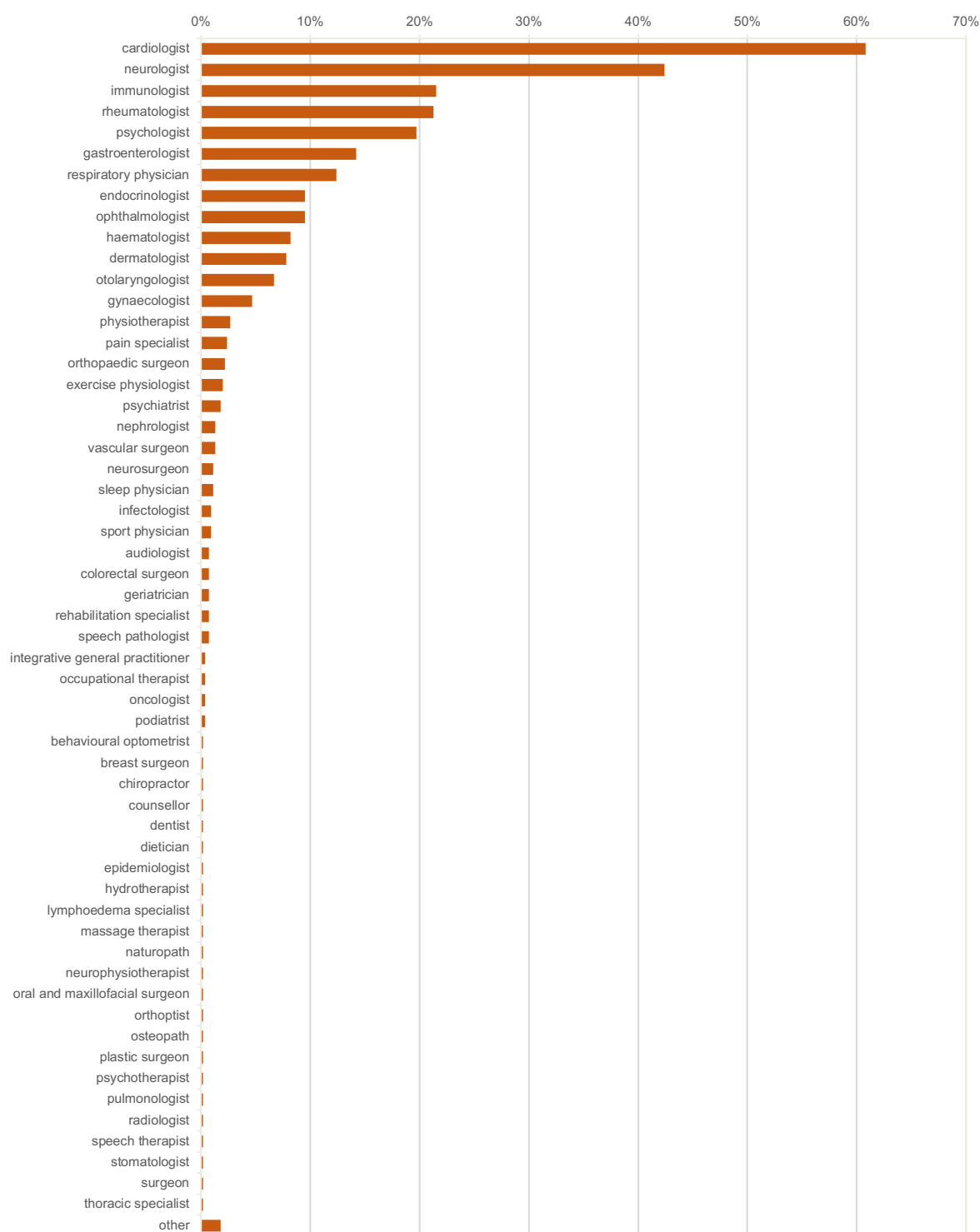


Figure 11. Specialists seen (including allied health professionals).

Of N₅₄₂ responses, 53.7% indicated they had received a formal medical diagnosis. Of these diagnoses, N₂₆₈ provided further details. We utilised Copilot to assist with analysing these responses:

These responses coherently cluster around several clear areas:

- Cardiac inflammatory and rhythm disorders (especially pericarditis, myocarditis, arrhythmias).
- Neurological and neuromuscular conditions (peripheral neuropathies, demyelinating diseases, autonomic dysfunction, central nervous system inflammatory syndromes).

- Systemic immune-mediated and autoimmune diseases (both organ-specific and systemic).
- Autonomic dysfunction syndromes (particularly postural orthostatic tachycardia syndrome (POTS) and dysautonomia).
- Chronic multisystem symptom syndromes (especially myalgic encephalomyelitis / chronic fatigue syndrome (ME/CFS), fibromyalgia, chronic pain syndromes).
- Hematologic and thrombotic disorders (including thrombocytopenia, thromboembolism, antiphospholipid-related conditions).
- Inflammatory and autoimmune gastrointestinal, endocrine, and connective tissue diseases.

We note that pericarditis is the single most common of these diagnoses.

The data also includes functional and psychiatric diagnoses. However, these are always coexisting with structural or inflammatory diagnoses, suggesting they are secondary diseases rather than the cause of such multisystem pathologies.

Copilot notes that the dataset describes a concentration of inflammatory, immune-mediated, autonomic, cardiac, and neurological diagnoses, frequently coexisting within the same individuals. Rather than isolated single-organ diseases, many responses suggest multisystem illness patterns involving immune dysregulation, neuroinflammation or neuropathy, cardiovascular inflammation or instability, and autonomic nervous system dysfunction.

Moreover, when combined with the above symptom responses, Copilot notes that consistency between symptom phenomenology and diagnostic frameworks strongly suggests respondents are describing recognisable medical syndromes affecting multiple interacting systems, rather than isolated or purely subjective symptom reporting.

N₄₄₉ indicated that the median time it took to receive these diagnoses was 1-3 months. The distribution of time to diagnosis is presented in Figure 12.

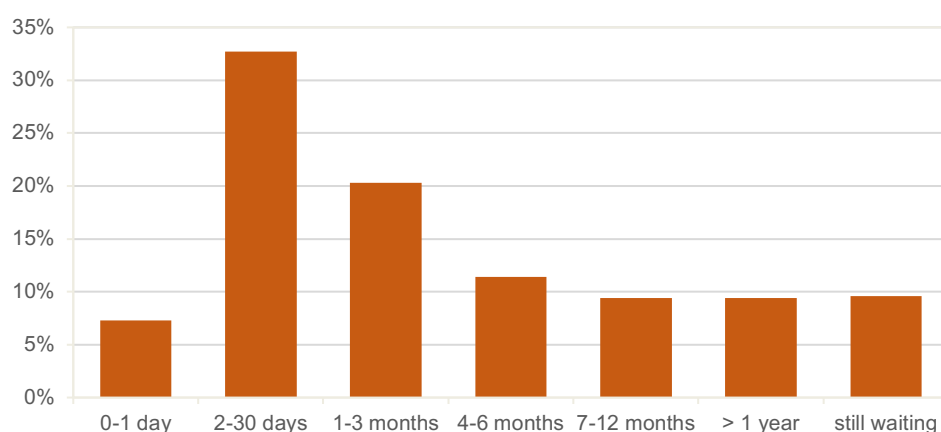


Figure 12. Time to receive diagnosis.

26.3% of N₅₂₀ respondents received a diagnosis that later proved to be incorrect.

We asked respondents to indicate test results, diagnosis or symptoms that seemed of particular significance. We used Copilot to assist with analysis of these N₃₂₀ responses.

The free-text responses varied widely, with different people choosing to articulate different aspects of their medical discovery. However, the pattern of cardiovascular, neurological, immunological, and autonomic disease featured heavily. Most notably, there was a recurrent disconnect between severe lived symptoms and either inconclusive or delayed diagnostic findings.

Within the variety of responses, the diagnostic tests most commonly noted as significant for the patient were cardiac MRIs, inflammatory/coagulation markers (CRP, D-dimer), and MRI-based neurological testing, and the notable diagnoses reflect what has already been articulated above.

A number of questions were asked about treatment approaches (prescription, alternative, self-medicated, etc.), and their perceived effectiveness towards improving symptoms. Responses were unstructured and highly diverse, which we analysed N₄₉₃ with assistance from Copilot:

Warning: treatment options described below have not been validated by us in any well-formed clinical trials, hence descriptions here cannot be taken as evidence of safety or efficacy. While some of these treatment approaches may have been described in published peer-reviewed studies, patients should consult with their healthcare teams before attempting any of them.

The most common prescribed medication was colchicine, which aligns with pericarditis being the most common diagnosis. Other medications that many have tried include prednisone/prednisolone, non-steroidal anti-inflammatory drugs (NSAIDs), low-dose naltrexone (LDN), beta-blockers, amitriptyline/endepr, proton pump inhibitors, and intravenous immunoglobulin (IVIG).

Most people also attempted non-pharmaceutical, allied health, and alternative health treatment approaches alongside conventional care, of which the most common were acupuncture, naturopathy, physiotherapy, myotherapy, chiropractic / osteopathy, hyperbaric oxygen therapy (HBOT), intermittent fasting, dietary changes, and Chinese medicine.

Overall, the interventions found to be most helpful were: magnesium, acupuncture, vitamins C and D, LDN, colchicine, ibuprofen and N-acetylcysteine (NAC). However, many people were either unable to articulate which treatments assisted them, or did not experience any notable improvement from any of them.

The least helpful treatments were more often the conventional pharmaceutical products, especially where respondents reported side effects from them. This includes most of the products also identified as most helpful for some people, highlighting highly individualised responses to these medications amidst a heterogeneous patient cohort.

When matched to the official diagnoses that respondents reported, the following patterns were observed:

- Pericarditis / myocarditis: colchicine was most helpful, steroids were sometimes helpful in the short-term, and responses to NSAID use were highly variable.
- Dysautonomia / POTS: electrolytes, rest/pacing and LDN were most helpful.
- Neuropathies, demyelinating syndromes: LDN, magnesium, and acupuncture were most helpful. However, when used, IVIG was almost always helpful. The range of neuropathic pain medication tried often came with side effects that negatively impacted perceived benefits.

- ME/CFS: rest/pacing, magnesium, intermittent fasting and acupuncture were most helpful. And decidedly unhelpful were antidepressants and clinical advice to “just push through”.

Overall, specifically targeted therapies towards clear diagnoses were helpful. However, it is important to note that in the vast majority of respondents who experienced improvement, helpful treatments led to symptom relief rather than cure.

Injury impacts

At the peak of symptom severity, 69.9% of N₅₂₅ respondents were unable to undertake *any* work, including domestic chores. In the month prior to respondents completing the Vaccine Injury Profile, this number had reduced to 22.7%. Figure 13 depicts estimated work capacity at symptom peak and in the month prior to completing the Vaccine Injury Profile.

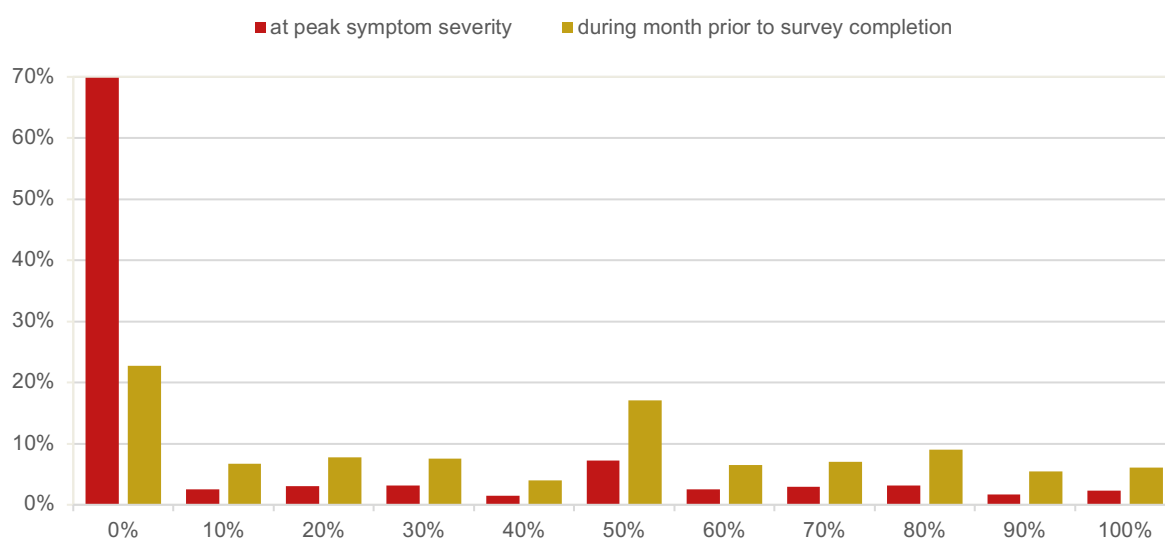


Figure 13. Estimated work capacity.

Figure 14 presents the distribution of how long it took N₄₉₂ respondents to be able to resume their normal activities and workload, with almost 40% of respondents still unable to undertake their normal tasks at the time of survey completion.

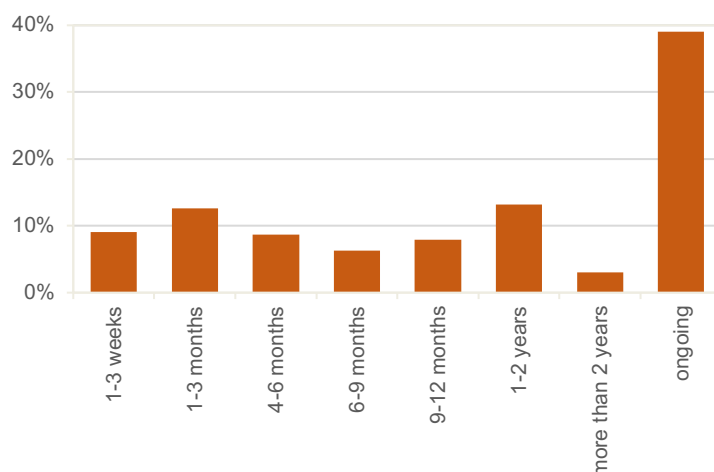


Figure 14. Time to be able to resume normal activities and workload.

Most respondents, N₄₈₄, indicated they continue to require additional assistance, primarily regarding medical care. Overall results regarding respondent needs are displayed in Figure 15.

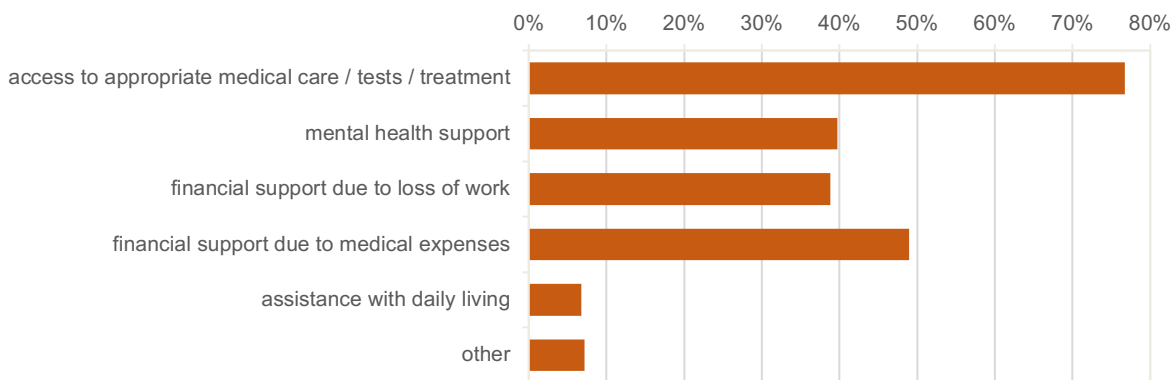


Figure 15. What respondents most need assistance with.

Financial issues

More than half, 54.9%, of N₅₂₅ respondents were not able to access any financial assistance, and only 7.24% did not require any. Others were able to access a variety of financial supports, including workplace entitlements, loans, and government programs. Figure 16 presents the range of financial assistance options that people have been able to access.

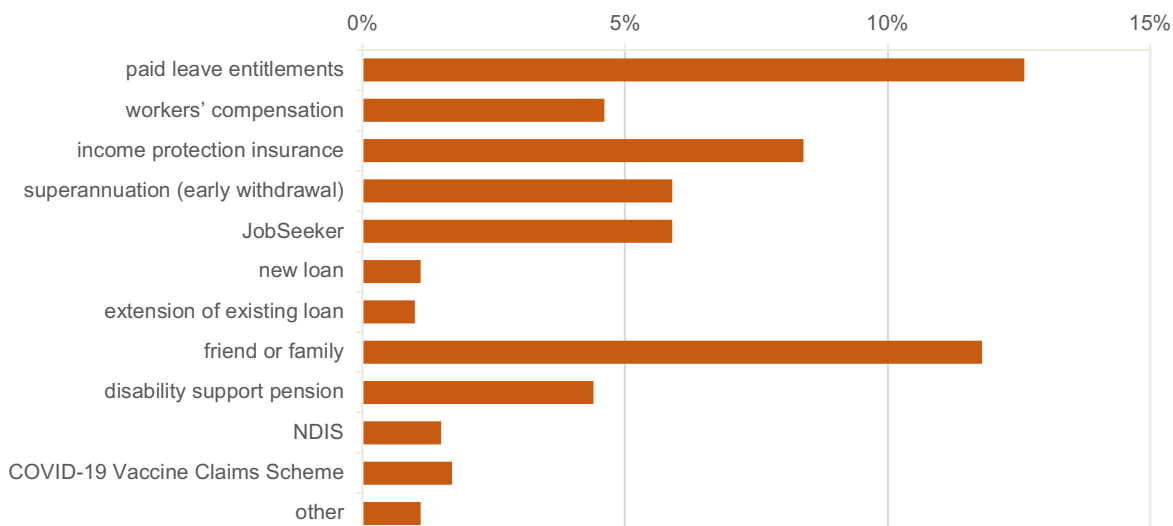


Figure 16. Avenues of financial assistance accessed.

Note that at the time of survey completion, a mere 1.7% reported successful claims with the Australian Government's COVID-19 Vaccine Claims Scheme.

Of those who were entitled to workers' compensation (N₈₁) (i.e. they were instructed to receive COVID-19 vaccination by their workplace), two thirds (66.7%) had issues with their workers' compensation claim. The majority of these issues were denial of claim (55%) or the workplace discouraging or not adequately informing or supporting claimants of their entitlements (11%). The majority of the remainder (30%) dealt with delayed processing of claims or inadequate reimbursement of medical expenses.

COVID-19 infection history

When asked about their COVID-19 infection status, 61.0% of N₅₁₅ respondents reported that they had experienced a COVID-19 infection. However, only 2.9% had contracted COVID-19 prior to their vaccinations, and only 7.0% contracted it *between* their COVID-19 vaccination doses.

Those who had not had COVID-19 were asked whether this was confirmed by SARS-CoV-2 antibody serology testing. 26.1% of N₁₈₄ reported a negative serology test, and the remainder had not done this test.

Medical history

51.5% of N₅₁₃ respondents report having pre-existing medical conditions at the time of their COVID-19 vaccinations, of which 75.4% report a worsening of those conditions following COVID-19 vaccination.

A wide range of pre-existing conditions were reported. The most prevalent were: asthma (12.9%), hypertension (10.6%), depression (8.7%), anxiety (7.2%), arthritis (5.3%), fibromyalgia (4.9%), ME/CFS (4.9%), migraines (4.6%), PTSD (4.6%), and diabetes (4.2%).

Personal stories

At the end of the Vaccine Injury Profile, we asked, “Tell us your story in your own words”, to which N₄₁₆ respondents contributed varying aspects of their journeys. All stories were read by COVERSE personnel and were manually sanitised: spelling was corrected, health terminology was harmonised (where appropriate), and all personally identifying information was removed (including names, exact dates, locations, employers, etc.). The median length of each story was 416 words (IQR 99-508), or 3 paragraphs (IQR 1-7). Copilot was utilised on the sanitised versions to augment human review and analysis:

Despite individuals’ stories revealing a broad range of complaints, largely reflecting quantitative responses described above, most striking was just how similar the vast majority of these narratives were, despite representing a diverse population.

The following is an accurate reflection of the journeys described by a clear majority (60-90%) of patients:

1. Previously extremely fit, active, healthy, and engaged in their personal and professional lives, and previous high levels of trust in public vaccination.
2. Mandated, or otherwise felt pressure or coerced, into getting vaccinated, for reasons other than self-concern about contracting COVID-19.
3. Reaction began very quickly, within 7 days and often within hours.
4. Dramatic and highly disabling symptoms, notably crushing fatigue, cognitive impairment, autonomic dysfunction, commonly resembling what is described as Post COVID-19 Vaccination Syndrome (PCVS).
5. Where diagnoses align with conditions recognised by government drug agencies, almost all of these patients suffer additional highly disabling symptoms and conditions not described by those agencies, commonly aligning with PCVS.
6. Symptoms are neither mild nor self-resolving.
7. Repeated presentation to hospital emergency, resulting in little meaningful diagnostic evidence in support of symptoms.

8. Outright gaslighting and psychologisation from medical professionals, or acknowledgement of potential vaccine reaction but unwillingness to document it.
9. No follow-up or investigation from pharmacovigilance authorities, leaving patients (and their treating physicians) without answers.
10. Advised and pressured into getting further COVID-19 vaccinations, while exemption from further vaccination was impossible to obtain.
11. Additional doses dramatically worsen symptoms.
12. No compensation.
13. More than 15% contemplated ending their lives, and up to half faced extreme despair.
14. Contracting COVID-19 leads to a significant flare of symptoms, but this is usually temporary.
15. Loss of previous life, loss of function, loss of independence, financial loss, ongoing impacts on family.
16. Erosion of trust in government, drug regulators, health systems, and doctors.
17. Wish to return to their previous lives, be believed, receive meaningful treatment, and obtain accountability and justice.

Below is a collection of short quotes from the stories that convey the situations and trauma these people have experienced:

- “I’ve gone from a healthy adult to someone that struggles with life.”
- “I feel that I’m alone in this.”
- “My life feels like it’s slipping through my hands.”
- “It’s been the biggest regret of my life.”
- “I thought I was going to die.”
- “My burning passion for life had been almost completely snuffed out.”
- “On bad days I do pray that my life will end soon.”
- “I have lost my family, livelihood and friends.”
- “I’m at the point of suicide.”
- “It has ruined my life.”
- “I just want my old life back.”
- “They have taken away my dignity.”
- “I know I am dying because of this.”
- “My life finished after the vaccination.”
- “I don’t think I will ever get better.”
- “I’ve been suffering through a living hell.”
- “The vaccine destroyed my life.”
- “I wish that one day I could wake up and be my normal, physical sporty self again.”

Comments

By undertaking this survey, **COVERSE** has done more than any government or health organisation in Australia to track and document the stories and trajectories of Australians who have been negatively impacted by COVID-19 vaccines. The people, their data and stories, depicted here represent a fraction of those Australians who have suffered harms from the COVID-19 vaccines.

Through our direct engagement with, literally, thousands of Australians who have suffered serious adverse reactions following their COVID-19 vaccinations, we note that the data and stories captured in the Vaccine Injury Profile form an accurate picture of the broader situation that our constituency has faced and continues to face.

At the time of the preparation of this document, the vast majority of Australians harmed by the COVID-19 vaccines have **not** been compensated by the Australian Government. While some 90% of *those who applied* for compensation from the COVID-19 Vaccine Claims Scheme have had their claims rejected (this includes a significant number who withdrew their claims based on advice from Services Australia prior to completing the full assessment), we note that a far larger number did not apply. The reasons people did not apply, despite extensive medical evidence of suffering from a COVID-19 vaccine-caused condition, include the excessive administrative burden of the Scheme, medical conditions not being listed as compensable by the Scheme, not fulfilling non-medical administrative aspects of the scheme, or simply not having the energy to pursue a claim due to significant and ongoing symptom burden.

It is our view that the consistency observed amongst the symptom patterns and patient stories point to a potentially significant safety signal. This is especially articulated when observing the same patterns emerging from independent surveys of other patient organisations, including in [Germany](#), [Japan](#), the [UK](#), and the [USA](#).

Australia's Therapeutic Goods Administration (TGA) lists more than 140,000 AEFI reports (of which more than 22,000 are classified as *serious*), which represents almost ¼ of all reports (for all vaccines, drugs, supplements, and medical devices) recorded by the group since 1971. While the TGA does not make causality determination against any of these individual reports, the data and stories we have captured suggests that widespread structural failures (including outright gaslighting) compounded with the lack of follow-up or assessment of AEFI reports, has left the TGA (and similar pharmacovigilance organisations around the world) blind and/or ignorant to the realities of the people who suffer these harms. This significant shortcoming not only affects the lives of those who suffer from these harms, it also impacts broader discussions of vaccine safety in the community. In fact, while governments lay blame for declining vaccination rates on "misinformation" (and, occasionally, on "vaccine fatigue"), it is abundantly clear to us that lack of acknowledgement of, research into, and compensation for, people whose lives have been destroyed by very serious harms caused by the COVID-19 vaccines plays a significant role in this decline.

We urge the Australian Government to:

- Follow up every serious AEFI report to track symptom trajectory, diagnoses, emerging evidence, and mortality.
- Refer all cases of potential or unclear causality to advanced medical research facilities for detailed assessment and research, in order to try and find answers for those patients and to improve understanding of vaccine-caused diseases.

- Compensate every single case of serious adverse events following vaccination for which there is no evidence for a more likely cause, including adequate coverage for both formal and informal medical costs, as well as generous pain and suffering owing to the fact that these people would still be healthy today if it were not for a vaccine that was part of a major public health initiative (and in many cases unnecessarily mandated).
- Issue a formal apology in Parliament, and on behalf of Australia, to those people who have suffered harms from vaccines and been dismissed, gaslit, derided, and denied treatment and compensation.

For those interested in the evolving science around COVID-19 vaccine injuries, **COVERSE** (in collaboration with our international partners) maintains a curated database of peer-reviewed medical and scientific articles, which is publicly accessible at: coverse.org.au/science.

COVERSE's submissions to public inquiries are all freely available and accessible at coverse.org.au/submissions, which serves as a historical record for future generations to look back and see how our governments and parliaments abandoned citizens harmed through the public COVID-19 vaccination campaigns.

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